Borderline and Other Personality Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

Brian Campbell, Ph.D.

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1. What it is

A personality disorder is a long-standing, inflexible pattern of thinking, feeling, relating, and impulse control that differs markedly from cultural expectations, begins by adolescence or early adulthood, and causes distress or impairment across many situations (American Psychiatric Association [APA], 2022). DSM-5-TR groups ten personality disorders into three clusters: **A** (odd or eccentric: paranoid, schizoid, schizotypal), **B** (dramatic or erratic: antisocial, borderline, histrionic, narcissistic), and **C** (anxious or fearful: avoidant, dependent, obsessive-compulsive).

How common: across 46 studies in 21 countries, 7.8% of adults had at least one personality disorder (Winsper et al., 2020).

Borderline personality disorder (BPD) is the most studied and the focus of this review. It involves instability in emotions, relationships, and self-image, with impulsivity and often self-harm (Gunderson et al., 2018).

Hope for recovery: over 16 years, 78% to 99% of patients with BPD achieved symptom remission, though fewer (40% to 60%) achieved full recovery with good social and work functioning (Zanarini et al., 2012). BPD is not a life sentence.

2. Core symptoms to watch for

DSM-5-TR criteria for BPD, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive pattern, beginning by early adulthood, with **5 or more** of:

- Frantic efforts to avoid real or imagined **abandonment**.
- Unstable, intense **relationships** that swing between idealizing and devaluing.
- Unstable **self-image** or sense of identity, and chronic feelings of **emptiness**.
- **Impulsivity** in at least two self-damaging areas (such as spending, sex, substances, reckless driving, binge eating).
- Recurrent **suicidal behavior, gestures, threats, or self-harm**.
- Intense, reactive **mood swings** lasting hours to a few days, and intense, hard-to-control **anger**.
- Brief, stress-related **paranoid thinking or dissociation**.

Screening: the McLean Screening Instrument (MSI-BPD) was developed as a brief self-report screen for BPD (Zanarini et al., 2003). Diagnosis requires a full clinical assessment.

Suicide risk: across 8 studies of 1,179 patients with BPD, 94 died by suicide, far above the general-population rate (Pompili et al., 2005). Assess self-harm and suicide risk at every stage, and build a safety plan.

When counselors must refer (general clinical consensus): BPD needs a structured, specialist-supported approach. Refer for psychiatric evaluation for suicidality, serious self-harm, psychosis, or substance dependence, and seek consultation when treating BPD.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in borderline and other personality disorders. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
MSI-BPD (McLean Screening Instrument for BPD)	Quick screen for borderline personality disorder.	Self-report, 10 yes/no items	No (free)	None beyond general clinical training	Original article

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
BSL-23 (Borderline Symptom List-23)	Tracking the severity of borderline symptoms during treatment.	Self-report, 23 items	No (free)	None beyond general clinical training	Original article
ZAN-BPD (Zanarini Rating Scale for BPD)	Clinician rating of change in borderline symptoms.	Clinician interview, 9 items	No (free)	★ Clinical interviewing skill; training recommended	Original article
SCID-5-PD (Structured Clinical Interview for DSM-5 Personality Disorders)	Diagnosing all ten DSM-5 personality disorders, with an optional self-report screener.	Semistructured clinician interview	Yes	★ Clinical interviewing skill; training recommended	APA Publishing
PID-5 (Personality Inventory for DSM-5)	Dimensional personality trait profile (negative affect, detachment, antagonism, disinhibition, psychoticism); brief and informant forms.	Self-report; 220-item full or 25-item brief form	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
PAI (Personality Assessment Inventory)	Broad clinical and personality assessment; includes a borderline features scale.	Self-report, 344 items, about 50 to 60 minutes	Yes	★ Level C	PAR
MCMI-IV (Millon Clinical Multiaxial Inventory-IV)	Adults in treatment; personality patterns alongside clinical syndromes.	Self-report, 195 true/false items, about 25 to 30 minutes	Yes	★ Level C	Pearson
MMPI-3 (Minnesota Multiphasic Personality Inventory-3)	Broad personality and psychopathology assessment, with validity scales for over- and under-reporting.	Self-report, 335 true/false items, about 25 to 50 minutes	Yes	★ Level C	Pearson
C-SSRS (Columbia Suicide Severity Rating Scale)	Screening and assessing suicide risk; brief screener and full versions.	Clinician- or staff-asked questions, a few minutes	No (free)	Free online training offered and recommended	Columbia Lighthouse Project

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: structured psychotherapy, especially dialectical behavior therapy (DBT), helps; well-organized general psychiatric care can work about as well. Medication does not treat BPD itself.

Evidence overview (honest effect sizes):

- **Psychotherapy overall:** in a Cochrane review of 75 trials (4,507 people), psychotherapy reduced BPD severity compared with usual care (SMD −0.52, moderate-quality evidence), and modestly reduced self-harm and suicide-related outcomes (Storebø et al., 2020). Another meta-analysis of 33 trials found small-to-moderate effects; DBT and psychodynamic approaches had the clearest support, and bias inflated many results (Cristea et al., 2017).
- **DBT** (a cognitive-behavioral treatment teaching mindfulness, emotion regulation, distress tolerance, and relationship skills): compared with expert community therapists, DBT halved suicide attempts and reduced dropout and hospital use over 2 years (Linehan et al., 2006). However, **general psychiatric management** did about as well as DBT in a 1-year trial (McMain et al., 2009).
- **Standard CBT:** adding CBT to usual care did not reduce the combined rate of suicidal acts, hospital admissions, and emergency visits, though suicidal acts alone did decline (Davidson et al., 2006).
- **Other personality disorders:** among 323 patients, schema therapy (which targets long-standing core beliefs about self and others) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014).
- **Medication (referral notes):** in 46 trials, no medication improved BPD's primary outcomes more than placebo (Stoffers-Winterling et al., 2022). Medication may still treat co-occurring depression, anxiety, or psychosis.

Cognitive-behavioral work with BPD: how it is used (Linehan et al., 2006; Bamelis et al., 2014):

- **Validate first, then change.** Accept the person's pain while building new skills.

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- **Build skills:** mindfulness, naming and regulating emotions, tolerating distress without self-harm, and asking for needs without damaging relationships.
- **Identify and challenge core beliefs** such as "I am unlovable," "Everyone will abandon me," "I am bad," and "I can't stand this feeling."

5. Religion, spirituality, and Christian integration: what the evidence shows

Spirituality and forgiveness are often lower in BPD. Among internal-medicine outpatients, people with more borderline symptoms reported lower spiritual well-being (Sansone et al., 2012), and in a sample of 307, lower forgiveness on many measures (Sansone et al., 2013).

Forgiveness grows with recovery. Over 20 years, patients with BPD began about 70% lower than comparison patients in acceptance and forgiveness, but these increased over time, and recovered patients reported about three times the levels of non-recovered patients (Zanarini et al., 2020).

Intervention study. Among 40 adults in outpatient DBT, a **forgiveness module** increased forgiveness and reduced attachment insecurity and psychiatric symptoms, gains kept at 6 weeks; the authors call for a randomized trial (Sandage et al., 2015). Clinicians are also advised to assess and sensitively engage spiritual and religious experiences within standard BPD care (Unruh, 2025).

Honest summary for teaching:

1. DBT and other structured therapies, with good general psychiatric management, are the evidence base. Faith integration supports them; it does not replace them.
2. Forgiveness and acceptance tend to grow as people with BPD recover, and a forgiveness module within DBT showed promising early results.
3. No randomized trial of Christian-integrated treatment for BPD exists yet. Religious experiences can steady or destabilize these clients, so assess them carefully.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within DBT-informed, structured care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for core beliefs that drive abandonment fears, rage, and self-harm, often in spiritual form: "God will leave me like everyone else," "I am too bad to be loved," "I can never be forgiven" (Sansone et al., 2013).
- **Challenge (clinical).** Validate the pain, then test the evidence for all-or-nothing beliefs about self and others; practice skills before trying to change long-held beliefs (Linehan et al., 2006). **Possible cognitive errors** to watch for, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
    - **All-or-nothing thinking** — no middle ground. *Feeds:* unstable relationships. *Sounds like:* "You're either for me or against me."
    - **Emotional reasoning** — I feel it, so it must be true. *Feeds:* emptiness and unstable self-image. *Sounds like:* "I feel empty, so I am nothing."
    - **Fortune telling** — predicting a bad outcome. *Feeds:* fear of abandonment. *Sounds like:* "He'll leave me like everyone else."
    - **Mind reading** — assuming I know what others think. *Feeds:* anger and suspicion. *Sounds like:* "They secretly hate me."
    - **Global labeling** — one mistake becomes who I am. *Feeds:* shame. *Sounds like:* "I'm too bad to be loved."
    - **Catastrophizing** — expecting the worst possible outcome. *Feeds:* crises and self-harm. *Sounds like:* "I can't survive this feeling." Always ask about self-harm and suicide when you hear this.
- **Questions to dispute the belief.** Have the client choose questions from the Questioning False Beliefs form that "speak to" the belief. Selected questions that fit the errors above:
    - "Am I keeping things in perspective?" *For:* all-or-nothing thinking.
    - "What's the actual evidence for my belief?" *For:* emotional reasoning.
    - "Can I really predict the future?" and "Does God still love me despite my faults or limitations?" *For:* fortune telling and "God will leave me".
    - "Are there other ways of looking at this situation?" *For:* mind reading.

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    - "Are my sins really too big for the God of the universe to forgive?" *For:* global labeling and "I can never be forgiven".
    - "Am I underestimating my ability to cope?" *For:* catastrophizing.
- **Replace (pastoral).** Offer Scripture of God's permanent acceptance and of forgiveness:
    - "Though my father and mother forsake me, the LORD will receive me." (Psalm 27:10). For the fear of abandonment.
    - "…for God has said: 'Never will I leave you, never will I forsake you.'" (Hebrews 13:5)
    - "Accept one another, then, just as Christ accepted you, in order to bring glory to God." (Romans 15:7). Acceptance received, then given.
    - "Bear with one another and forgive any complaint you may have against someone else. Forgive as the Lord forgave you." (Colossians 3:13). Forgiveness as a gradual process, not a demand to reconcile with someone unsafe (Sandage et al., 2015).
- **Faith community (clinical).** Help the church offer steady, consistent relationships with clear, kind limits. Unpredictable closeness and rejection can worsen BPD symptoms.
- **Ministers:** keep healthy boundaries, take every mention of suicide or self-harm seriously, and refer to a qualified clinician.

7. Quick classroom takeaways

- BPD involves 5 or more of 9 criteria marked by instability in emotion, relationships, and identity, with impulsivity and self-harm. Suicide risk is high; assess it at every stage.
- DBT and structured care work; standard CBT alone is less clear; medication does not treat BPD itself. Most patients remit over time.
- Forgiveness and acceptance grow with recovery. Replace "everyone will leave me" with the truth that God will never forsake His children.
- Screen for BPD with the free MSI-BPD, track change with the BSL-23, and confirm with a structured interview such as the SCID-5-PD.

Sources note

All 18 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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